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THE MEDICAID DOLLAR PROJECT

$100 Medicaid Dollars

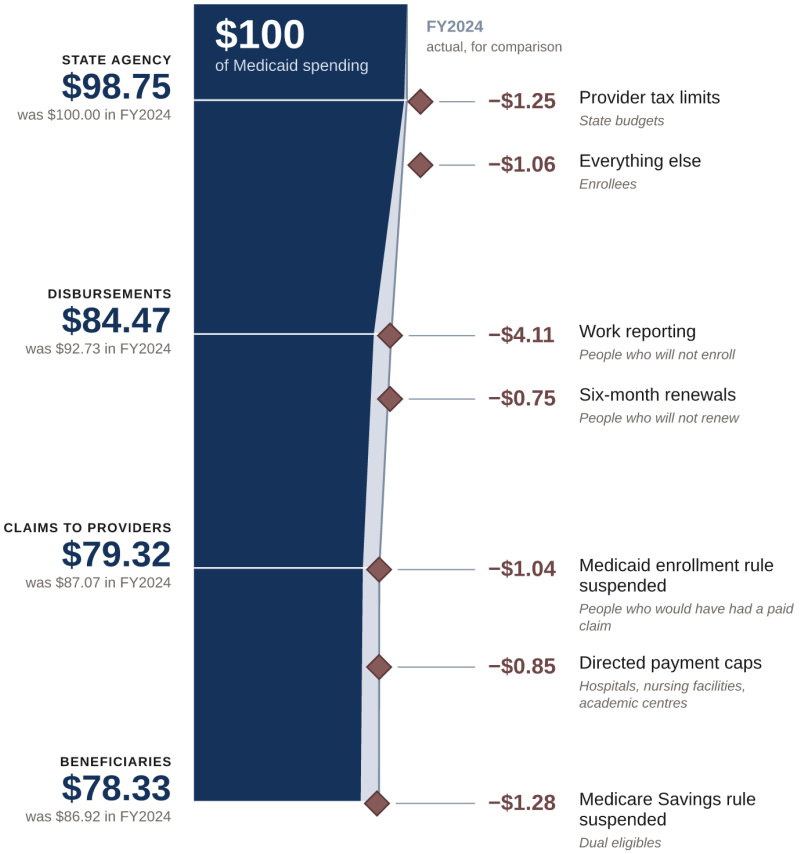
The national view, and what P.L. 119-21 will do to it by fiscal year 2030. Every figure is dollars per $100 of Medicaid spending.

PREPARED FOR THE MHPA BOARD OF DIRECTORS

Jamey Harvey · Agilian LLC

Where the money stops

$100 of Medicaid, and the seven cuts of P.L. 119-21



P.L. 119-21 removes $10.34 of every $100. Overhead is assumed to fall with it, returning $1.74.

$8.60 less reaches care.

FY2024 actual against FY2030 projected under prior law. Every figure modelled from CBO and CMS-64.

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Ten sections, one idea each.

1	Introduction	$86.92 reaches care today
2	The baseline, before the law	$13.08 runs the program
3	The national baseline, end to end	$51.34 arrives through a plan
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5	What the law does	Work reporting is 40% of it
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$86.92 reaches care today. $78.33 will by 2030.



Of every $100 of Medicaid spending, $86.92 arrives as a health service today. Under P.L. 119-21 that becomes $78.33. The rest runs the program: state and plan administration, Medicare premiums returned to the federal government, retained earnings, and a small amount of documented fraud.

Everyone in Medicaid has heard the national number. None of it tells you what happens at your own position in the ecosystem. This deck does, in one currency: dollars per $100.

Every figure is read from the model at build time. The ledger conserves to the cent in both years.

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$13.08 runs the program — $5.65 of it through plans.



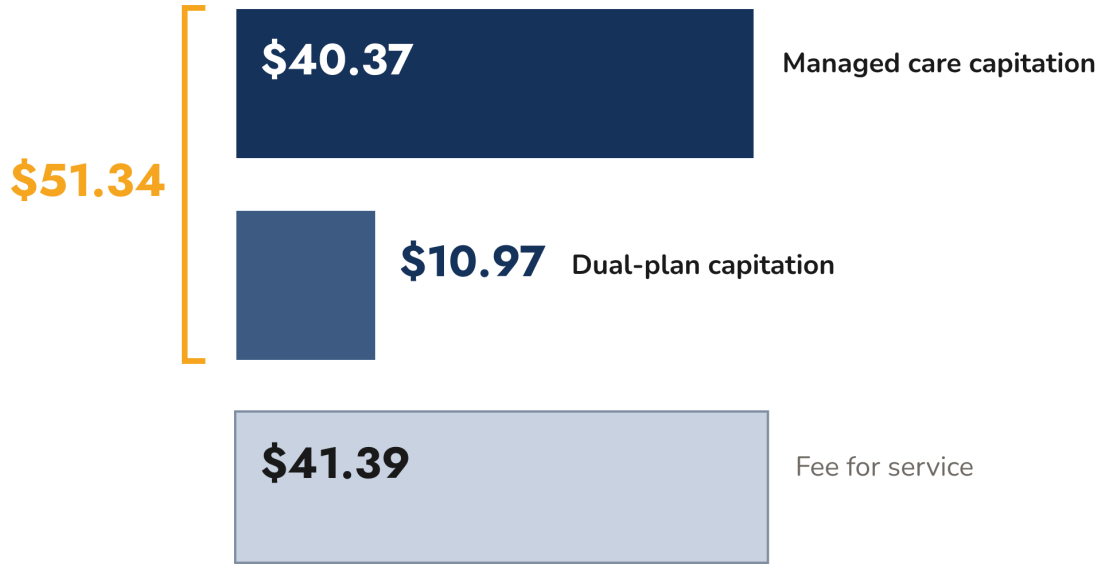
Before P.L. 119-21 does anything, this is the hundred a plan already works inside. State and program administration takes $7.27. Plan administration and earnings is $5.65. Documented fraud is $0.15.

This is the starting position every provision in the next eight slides is applied to.

FY2024 actuals. CMS-64 national totals and MACPAC MACStats, February 2026.

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$51.34 of every $100 arrives through a plan.



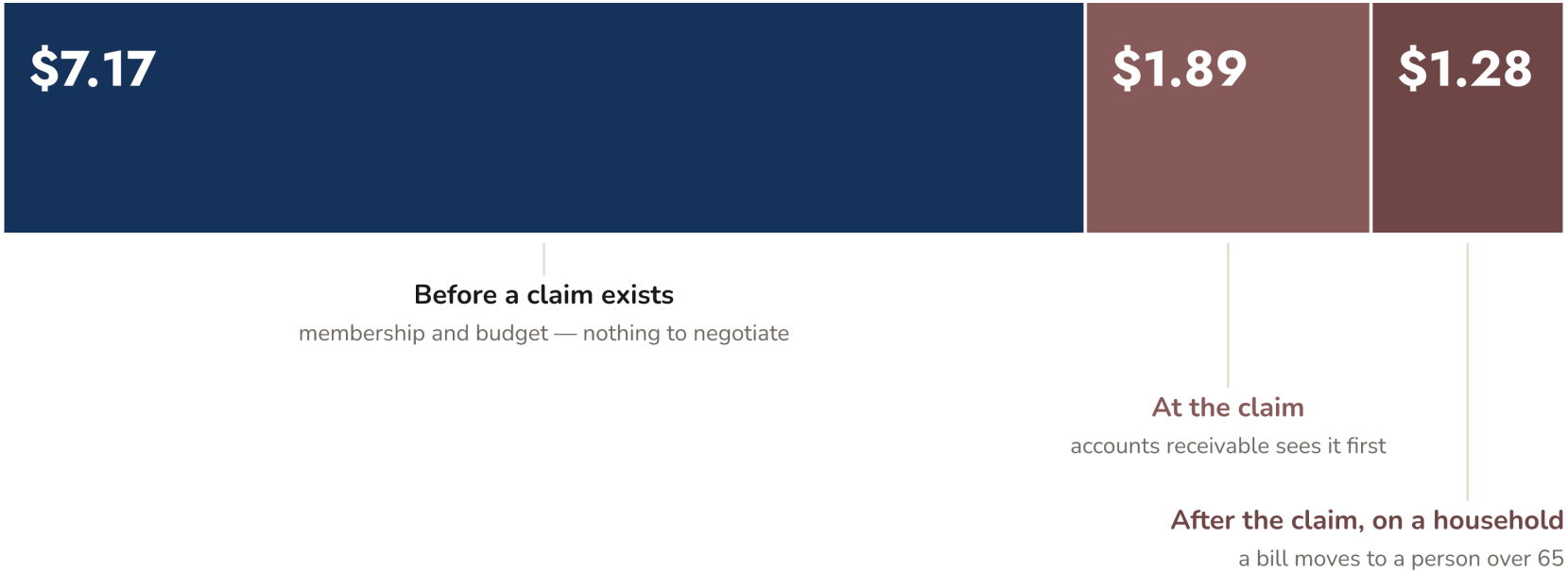
Of the $92.73 a state agency disburses, $40.37 is managed care capitation and $10.97 is dual-plan capitation. The remaining $41.39 is fee for service, which passes the payer phase untouched because no plan holds it.

A capitated dollar is committed the day the month starts. A fee-for-service dollar is committed only when a claim is filed. That is why an enrollment change and a rate change do not feel the same.

Dual-plan capitation is carved out because the plans are different, the populations are more expensive, and the law treats them differently.

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$7.17 of the $10.34 leaves before a claim exists.



Seven provisions remove $10.34 per $100 by FY2030. Where each one leaves decides who can do anything about it.

$7.17 goes before a claim exists — membership and budget, with nothing to negotiate. $1.89 leaves at the claim, which is what a provider's accounts receivable sees first. $1.28 leaves after the claim, onto a household.

Measured at FY2030, where 62.6 percent of the ten-year effect is at full force and every provision has started. FY2030 will not be the worst year.

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Work reporting is 40% of the law.



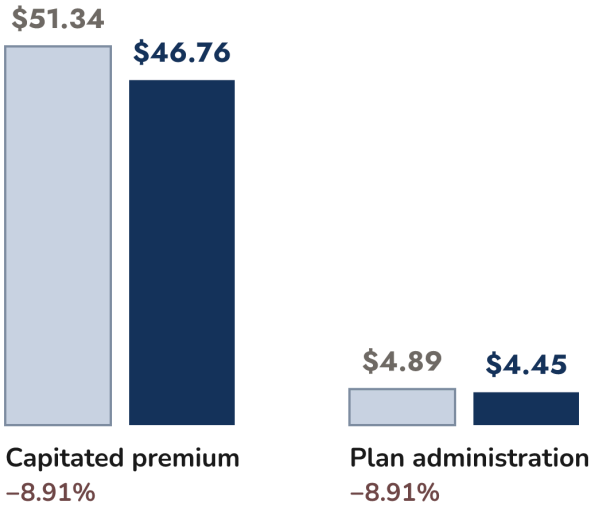
The seven provisions, sorted by size. Work reporting alone is $4.11 of the $10.34 — more than the next two together — and it lands at disbursements, before a claim is ever filed.

Each provision leaves at its own phase for its own reason. Work reporting hits when a state stops disbursing for people who lose coverage to paperwork. Directed payment caps hit only after a claim is filed, which is why they land on a provider's books rather than a state's budget.

Plain-language names lead. Section numbers appear parenthetically at first use in the white paper.

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Same work, less pay.



What does not fall

- Network management
- Rate setting
- Regulatory reporting

0.00%
unchanged

No provision in P.L. 119-21 addresses managed care directly. Premium falls 8.91 percent because fewer people are enrolled. Plan administration falls with it, from $4.89 to $4.45, because load sits inside the capitation rate as a percentage of premium. A smaller base yields a smaller administrative dollar with nothing negotiated between payers and states.

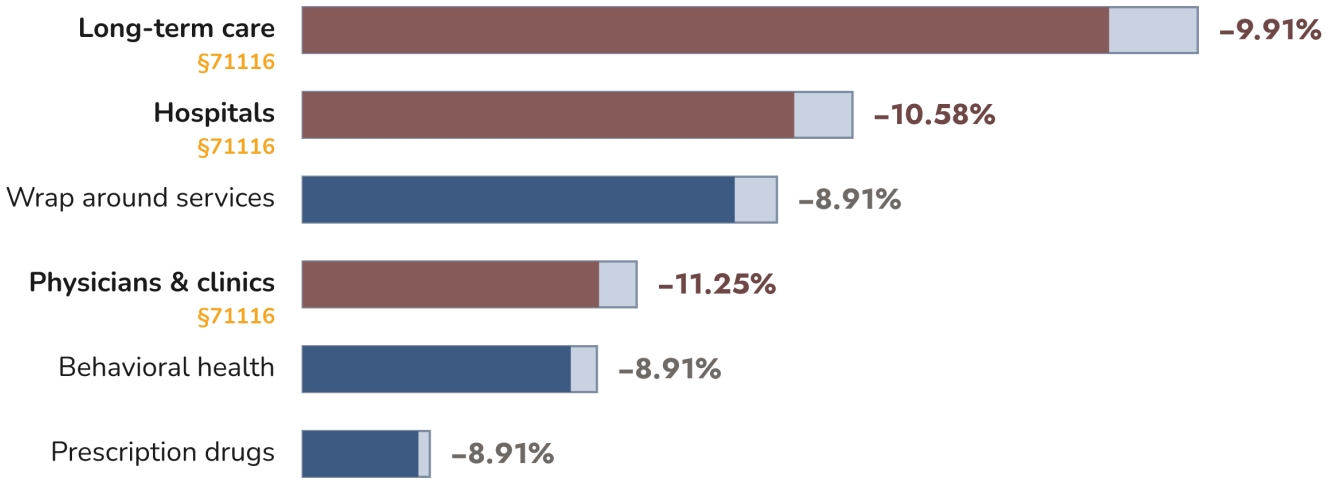
Network management, rate setting and regulatory reporting do not shrink because membership does. Risk pool mix will shift, because eligibility rules remove the members easiest to disenroll, not the members most expensive to cover.

The claims phase is different in kind. Under prior law no money left at claims. Under P.L. 119-21, $1.89 leaves here — the only money in the lifecycle removed after the point where a service could already have been rendered.

Where a plan holds directed payment arrangements the cap is an instruction to carry out, but the plan rather than the state has to renegotiate with providers.

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Three services fall faster because §71116 names them.



Long-term care, hospitals, and physicians and clinics carry the directed payment cap on top of the general contraction, in proportion to how much of each service arrives through a directed payment.

Wrap around services, behavioral health and prescription drugs fall at 8.91 percent. Three services landing on an identical rate to two decimals is the absence of an explanation, not the presence of one.

The rate is a national average. Actual exposure depends on a state's expansion status, its managed care penetration, and an organization's own payer mix and directed payment position. Two hospitals in neighboring states can face materially different outcomes from the same statute.

The six services sum to claims paid: $87.07 in FY2024 and $78.47 in FY2030.

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The new adult group carries most of the loss. Dual eligibles are left holding an unpaid bill.

$5.43

of the $8.60 the law takes from people falls on the new adult group alone.

88% of it is coverage, not rate.

Not a smaller service — the end of enrolment.

§71119 and §71107 reach this group and no other.

$1.28

stops being paid by Medicaid and starts being paid by a dual eligible.

No provider loses revenue.

No service is cut. A bill moves.

The Medicare premium does not stop being owed.

Nothing else in the law is concentrated like this. §71119 and §71107 name the population enrolled under §1902(a)(10)(A)(i)(VIII) and reach no other, which is why this category moves by more than a quarter while every other category moves by less than five percent.

Of the seven provisions, six reduce what somebody is paid. One converts a Medicaid payment into a household obligation — the clearest statement in the ledger of what “removed from Medicaid” can mean.

Coverage is somebody dropped and the whole of their care goes to zero. Rate is the same person, still enrolled, with their provider paid less. No individual loses a share of their own care.

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A national average is not a local condition.

The state view

Expansion status, managed care penetration, provider tax structure and directed payment exposure vary enormously, and they compound rather than cancel. The state editions carry the same eight phases and the same five anchors.

A District of Columbia edition is furthest along. Tell us which state you need.

Where the removed dollars land

This analysis sizes what leaves the ecosystem and says where in the lifecycle it leaves. It stops short of claiming to know where each dollar lands.

Corrections and challenges are welcome and will be answered in print.

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Every figure carries a source.

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Jamey led the technology implementation of Obamacare for the District of Columbia and served as Deputy Chief Technology Officer for software and data for the District government before founding Agilian, a boutique firm helping Medicaid beneficiaries stay enrolled.

How this was built

The diagrams are produced by a model the firm wrote. The ledger, the geometry and the balance line are code, and every figure in this deck is read from that code at build time rather than typed into the prose. Claude, made by Anthropic, was used throughout as an analyst and drafting partner. No figure was supplied from memory. What this analysis claims, and what it declines to claim, was decided by the named authors.

Limits a board reader should know

- **Provider-level incidence beyond the three classes §71116 names.** Three of six services carry no service-specific finding, only an undifferentiated rate.
- **The provider tax lane understates its size.** It blends two mechanisms without a gross-up to total computable dollars.
- **Lever-to-node linkage is not shown.** A reader can see a service fall and cannot see which of the seven provisions did it.
- **Where a removed dollar lands is not claimed.** Care not sought, care written off, care paid out of pocket, or a cost a state absorbs elsewhere — no national source splits those four.

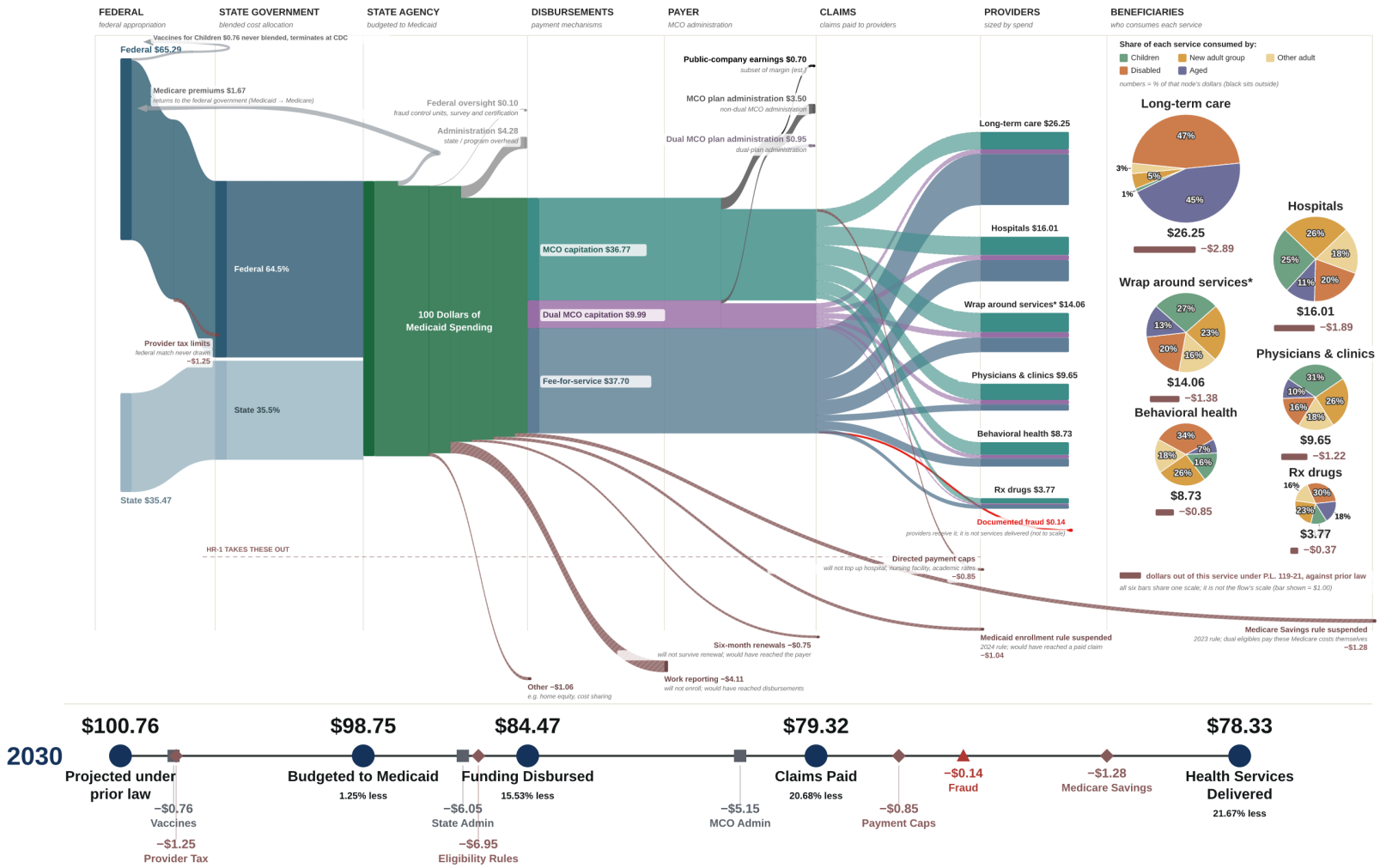
Sources

CBO Supplemental Cost Estimate for P.L. 119-21 Title VII Subtitle B Chapter 1, 28 October 2025, and the CBO letter of 5 March 2025 for the denominator. CMS-64 FY2024 national totals. MACPAC MACStats, February 2026, Exhibits 16, 17, 18 and 21. P.L. 119-21 itself.

The full white paper, visualization, and data model are accessible at https://agilian.com/100MedicaidDollarsProject

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$100 of Medicaid spending, FY2030.



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